INFORMATION TO KEEP WITH YOU

My complete treatment handbook

TCH → trastuzumab continuation

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
Intervals differ by treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00258
Version 9

Default: six 21-day TCH cycles with intravenous (IV) trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose subcutaneous (SC) trastuzumab alternatives; the team confirms the formulation and visit schedule.

Treatment stages

01 **Current combination**
21-day cycle

02 **Next treatment decision**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, chest symptoms, severe diarrhoea, dehydration, jaundice, unusual bleeding or sudden illness promptly. Do not wait for the next planned test.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

What each medicine does

Treat HER2-positive early breast cancer as part of a plan intended to reduce the risk of it returning.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell's division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

PLATINUM CHEMOTHERAPY

Carboplatin



Damages deoxyribonucleic acid (the cell’s genetic material) (DNA) inside cells

Carboplatin creates links in DNA, making it harder for cells to copy and repair their genetic instructions. Its dose depends on kidney function as well as the intended regimen.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Keep blood-test appointments. Fever, unusual bleeding, repeated vomiting or infusion symptoms need prompt advice.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Keep echo appointments throughout HER2 treatment. Report breathlessness, swelling, palpitations or a reaction; chemotherapy side effects are tracked separately.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

TCH is an adjuvant treatment after surgery for HER2-positive early breast cancer. Docetaxel and carboplatin provide chemotherapy without an anthracycline, with trastuzumab targeting HER2. Recurrence risk, lymph nodes, heart and kidney function, fitness and alternative treatments affect selection. Trastuzumab continues after chemotherapy to complete the agreed course.

What could change the choice?

Ask which cancer features, earlier treatments and health factors led to this recommendation, which alternatives were considered and when the decision will be reviewed.

Your treatment and the next step

Schedule

Default: six 21-day TCH cycles with intravenous (IV) trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose subcutaneous (SC) trastuzumab alternatives; the team confirms the formulation and visit schedule.

After this phase

Complete the selected chemotherapy and continue trastuzumab to the agreed total course, usually one year from the first dose; this is not an additional year after chemotherapy.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The phases and how they connect

Treatment stages

01 **Current combination**
21-day cycle

02 **Next treatment decision**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE
Current combination

21-day cycle

Default: six 21-day TCH cycles with intravenous (IV) trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose subcutaneous (SC) trastuzumab alternatives; the team confirms the formulation and visit schedule.

INDIVIDUAL PLAN
Next treatment decision

Individual plan

Complete the selected chemotherapy and continue trastuzumab to the agreed total course, usually one year from the first dose; this is not an additional year after chemotherapy.

Relative sequence, not equal lengths or booked dates. Your team confirms actual appointments and the active medicines.

BASELINE → TREATMENT → FOLLOW-UP

Tests and milestones through treatment

Columns show sequence, not equal durations or appointments. A test is only complete when performed and reviewed. Echo timing and cancer-imaging timing are different. Symptoms can bring any check forward.

Check	Surgery / baseline	TCH cycles 1–4	Around cycle 5 / week 12	Finish chemo → trastuzumab	Complete / follow-up
Blood tests	Baseline full blood count (FBC), renal and liver profile	Before each cycle; additional dose checks as prescribed	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities

Check	Surgery / baseline	TCH cycles 1–4	Around cycle 5 / week 12	Finish chemo → trastuzumab	Complete / follow-up
Echo / cardiac assessment	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / multigated acquisition (heart pumping) scan (MUGA) due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
Cancer imaging / markers	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Breast surveillance after early treatment
Surgery / radiotherapy	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects
Endocrine / bone health	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	dual-energy X-ray absorptiometry (bone density) (DEXA) when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00258 v9 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

TCH → trastuzumab continuation: the whole pathway

HER2-positive adjuvant breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Surgery / baseline	TCH cycles 1–4	Around cycle 5 / week 12	Finish chemo → trastuzumab	Complete / follow-up
Cancer medicines NCCP	Confirm diagnosis and exact regimen	See the dose-day map: use the current regimen and active phase	Echo due; treatment review	Finish chemotherapy; trastuzumab continues	Review benefit / tolerance and next plan
Surgery / radiotherapy EARLY	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects
Blood tests NCCP	Baseline FBC, renal and liver profile	Before each cycle; additional dose checks as prescribed	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities
Echo / cardiac checks NCCP · ESC	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / MUGA due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
Scans / tumour markers EARLY	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Breast surveillance after early treatment
Endocrine / DEXA BONE	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	DEXA when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

What each check is for

1

Blood tests

NCCP specifies baseline and regular FBC, liver and renal profiles; the team confirms the pre-dose checks for the active chemotherapy or trastuzumab-only phase. Verify kidney function for carboplatin dosing.

Check safety before the next dose.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

2

Heart tests

NCCP lists baseline echo/MUGA if clinically indicated, but requires cardiac eligibility assessment; the trastuzumab product information calls for baseline cardiac assessment. Repeat every 12 weeks during trastuzumab and at completion. Earlier checks follow symptoms or cardiac concern. Around cycle 5 is the first 12-week check if treatment stays on time.

Monitor pumping function and relevant rhythm risk.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

3



Cancer imaging

Baseline breast and nodal imaging, with staging tests appropriate to the diagnosis. Review response during preoperative treatment and before surgery. After local treatment, arrange breast surveillance; routine computed tomography (CT), positron emission tomography (PET) or tumour-marker testing is not the usual screening plan for someone without symptoms.

Assess response or follow-up needs.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

4

Bone health

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

Assess fracture risk where relevant.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan



STEP 1

Before the first visit

Confirm blood tests, heart assessment, active medicines, route and the consent discussion.



STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

When scans, heart checks and bloods happen

Agree who books each test, who reviews the result and what happens if a check is overdue.

Blood tests

NCCP specifies baseline and regular FBC, liver and renal profiles; the team confirms the pre-dose checks for the active chemotherapy or trastuzumab-only phase. Verify kidney function for carboplatin dosing.

Heart ultrasound and electrocardiogram (ECG)

NCCP lists baseline echo/MUGA if clinically indicated, but requires cardiac eligibility assessment; the trastuzumab product information calls for baseline cardiac assessment. Repeat every 12 weeks during trastuzumab and at completion. Earlier checks follow symptoms or cardiac concern. Around cycle 5 is the first 12-week check if treatment stays on time.

Scans and tumour markers

Baseline breast and nodal imaging, with staging tests appropriate to the diagnosis. Review response during preoperative treatment and before surgery. After local treatment, arrange breast surveillance; routine CT, PET or tumour-marker testing is not the usual screening plan for someone without symptoms.

Bone health and endocrine treatment

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

How surgery and radiotherapy fit

Surgery comes before this adjuvant chemotherapy. Radiotherapy, if indicated, is coordinated after chemotherapy and may overlap trastuzumab or later endocrine treatment when agreed.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Chemotherapy with HER2 treatment

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Chemotherapy with HER2 treatment

Treatment or check	Timing in this view
HER2 medicine · Day 1	 **Day 1** IV
Chemotherapy · Day 1	 **Day 1** IV
Administration reaction	 **Days 1–21** Report during or after any visit
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–21** Risk varies; fever needs prompt advice
Fatigue / mouth changes	 **Days 1–21** Variable timing and severity
Nerves / skin / nails	 **Days 1–21** Can build across doses
Heart / breathing symptoms	 **Days 1–21** Report on any day

Planned treatment: Default: six 21-day TCH cycles with IV trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose SC trastuzumab alternatives; the team confirms the formulation and visit schedule.

Heart checks: NCCP lists baseline echo/MUGA if clinically indicated, but requires cardiac eligibility assessment; the trastuzumab product information calls for baseline cardiac assessment. Repeat every 12 weeks during trastuzumab and at completion. Earlier checks follow symptoms or cardiac concern. Around cycle 5 is the first 12-week check if treatment stays on time.

Next phase: Complete the selected chemotherapy and continue trastuzumab to the agreed total course, usually one year from the first dose; this is not an additional year after chemotherapy.

> **Why these bands stay broad:** Treatment markers show the planned schedule. Symptom bands are broad possibilities, not measured risk curves; serious symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00258 v9 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

HER2-only continuation · source 21-day schedule

Day 1 → Day 21: a general pattern

HER2-only continuation · source 21-day schedule

Treatment or check	Timing in this view
Treatment · Day 1	 **Day 1** IV
Administration reaction	 **Days 1–21** Report during or after any visit
Heart / breathing symptoms	 **Days 1–21** Report on any day

Planned treatment: Default: six 21-day TCH cycles with IV trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose SC trastuzumab alternatives; the team confirms the formulation and visit schedule.

Heart checks: NCCP lists baseline echo/MUGA if clinically indicated, but requires cardiac eligibility assessment; the trastuzumab product information calls for baseline cardiac assessment. Repeat every 12 weeks during trastuzumab and at completion. Earlier checks follow symptoms or cardiac concern. Around cycle 5 is the first 12-week check if treatment stays on time.

Next phase: Complete the selected chemotherapy and continue trastuzumab to the agreed total course, usually one year from the first dose; this is not an additional year after chemotherapy.

Why these bands stay broad: Treatment markers show the planned schedule. Symptom bands are broad possibilities, not measured risk curves; serious symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00258 v9 · official regimen ↗

Between-treatment notes

Day 1–21: The current treatment cycle

Default: six 21-day TCH cycles with IV trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose SC trastuzumab alternatives; the team confirms the formulation and visit schedule. Your team confirms treatment dates and checks. Serious symptoms need advice on any day.

Support around the medicines

1 

The correct supportive prescription

Follow the script for the active medicine, route and treatment day. Do not carry chemotherapy anti-sickness medicines automatically into an antibody-only phase.

2

Keep heart checks on the plan

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

3

Plan for symptoms

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

Questions worth asking

Who arranges my tests?

Ask the team who books each test, who checks the result and who to contact if an appointment is overdue. This map does not book appointments.

What if I become unwell between visits?

Use the urgent contact plan for sudden illness, fever, chest symptoms, severe diarrhoea or poor fluid intake. Do not wait for a scheduled blood test or scan.

Try explaining the plan back: Which medicines are active now, which days do you receive or take them, and who is booking and reviewing your next tests?

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Carboplatin

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Allergic reaction, sometimes after previous exposure
- Kidney or electrolyte changes

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin combination

Active medicines: Docetaxel + Carboplatin + Trastuzumab

Emetogenic risk: High · carboplatin AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 Reconcile chemotherapy premedication steroids
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Individual contraindications, renal function and duration review
Dexamethasone	8 mg orally daily	Days 2–4 Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before treatment; minimum 3 doses pre-treatment unless contraindicated. If oral doses were missed, any 20 mg intravenous (IV) replacement is a consultant decision.
- Reconcile docetaxel premedication with the antiemetic steroid course and interactions; issue one coherent steroid prescription, not duplicate courses.
- Consider primary granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) using the source and individual risk; product and timing follow the prescribed plan.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Trastuzumab-only continuation

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine docetaxel or paclitaxel premedication or chemotherapy antiemetic course follows into antibody-only treatment.
- Phesgo or trastuzumab alone generally requires no routine antiemetic prophylaxis. Do not extend that statement automatically to an antibody-drug conjugate.
- Use the separately selected postoperative regimen for its blood tests, premedication, antiemetics and cardiac plan.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

TCH → trastuzumab continuation

Default: six 21-day TCH cycles with intravenous (IV) trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose subcutaneous (SC) trastuzumab alternatives; the team confirms the formulation and visit schedule.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart

Trastuzumab

Keep echo appointments; report breathlessness or swelling.

2 Hands / feet

Docetaxel

Report numbness or tingling before the next dose.

3 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

4 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

5 Fluid / skin / nails

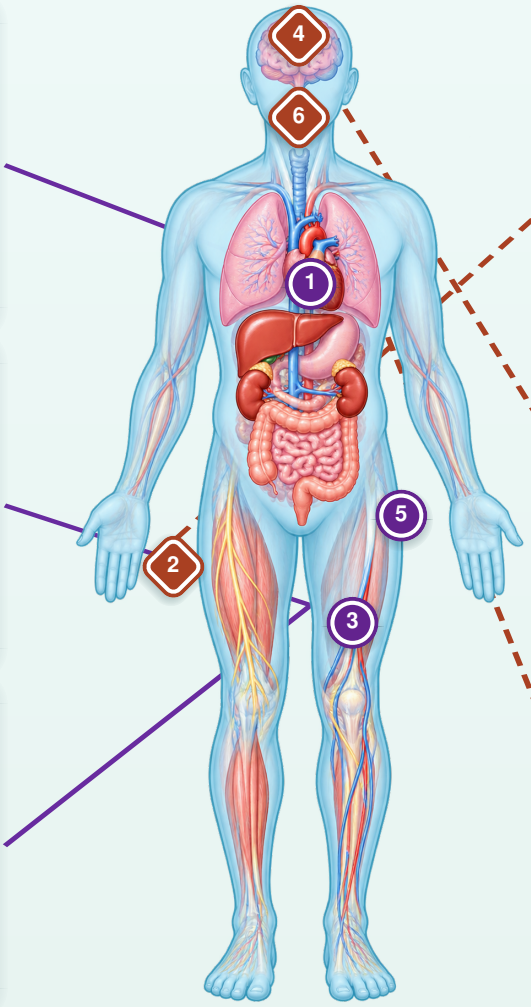
Docetaxel

Report swelling, painful skin or nail changes.

6 Mouth

Chemotherapy phase

Painful ulcers or difficulty drinking need advice.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Current regimen · reusable cycle record · copy this sheet for another cycle of the same phase.

Docetaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Carboplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999

Trastuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

TCH → trastuzumab continuation

NCCP 00258 v9 · 21-day cycle

Default: six 21-day TCH cycles with IV trastuzumab, then trastuzumab alone to complete one year total. The source also permits weekly IV or fixed-dose SC trastuzumab alternatives; the team confirms the formulation and visit schedule.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00258/ · no patient information.

Draft for review · SACTCheck v0.84.16

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00258 v9 · official regimen**
   https://healthservice.hse.ie/documents/6362/258_v9_TCH.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **EMA Herceptin product information**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
7. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
8. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
13. **EMA Perjeta product information**
    https://www.ema.europa.eu/en/documents/product-information/perjeta-epar-product-information_en.pdf